



Policy

Consent to Treatment

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Policy Details

This policy stipulates the consent requirements for patients who require an invasive procedure or treatment.

Invasive procedures or treatments, where there are known significant risks or complications or where the procedure requires general anaesthesia, major regional anaesthesia (epidural, spinal, axillary block) or intravenous sedation cannot be performed without consent.

This SCGOPHCG Consent to Treatment Policy is to be read in conjunction with the [DOH MP 0175/22 Consent to Treatment Policy and Procedure](#)

Risk Statement

Non-compliance with this will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Definitions

Consent	Informed consent is a person's decision, given voluntarily, to agree to a healthcare treatment, procedure or other intervention that is made: - Following the provision of accurate and relevant information about the healthcare intervention and alternative options available; and - With adequate knowledge and understanding of the benefits and material risks of the proposed intervention relevant to the person who would be having the treatment, procedure, or other intervention.
Competency	Competency, also called decision-making capacity, is a person's ability to make and communicate a decision to consent to medical treatment.
Capacity	Capacity is a person's ability to understand the nature and effect of their decisions and ability to make informed decisions and provide informed consent.
Health Practitioner	Health Practitioner - A person who is registered on a National Board in conjunction with the National Agency (AHPRA).

Policy Principles

Adult patients, or authorised representatives of patients have the right to determine what happens to their, or the person in their care's body. The provision of medical treatment without patient consent puts health professionals at risk of legal claims which may include trespass to the person and/or negligence except in cases where the law permits or requires treatment without consent.

All team members involved in a patient's perioperative care are responsible for ensuring the correct treatment or procedure is performed on the correct patient and at the correct site.

1. Procedure

A health practitioner has a legal obligation to fully inform the patient of the potential benefits of the treatment, medication, or procedure and of any known material risks that are inherent in a treatment, medication, or procedure, including the possibility that the treatment may be unsuccessful.

The delegating Medical Practitioner (MP) is responsible for ensuring that the delegated practitioner is competent to undertake the task. The delegating MP is to take reasonable steps to ensure that the patient has been properly informed and that a consent form has been completed.

Regardless of who obtains the consent, the most senior MP on the treating team is responsible for ensuring that consent to treatment has been obtained.

A Health Practitioner has the same responsibilities as a MP to obtain consent for the procedures they are performing.

The Office of Safety and Quality in Healthcare has a suite of [Procedure Specific Information Sheets](#) to assist Medical / Nurse / Allied Health Practitioners in outlining relevant risks to individual patients. Australian Commission on Safety and Quality in Healthcare publishes fact sheets for clinicians, including informed consent in health care. Refer to [Fact Sheet for clinicians- Informed consent in health care](#)

The registered nurse (RN) or an enrolled nurse (EN) giving care to the patient is responsible for ensuring that the patient's signed consent form is present in the medical record prior to administration of a pre-medication and/or preparation for transfer to the operating theatre suite/procedural unit, diagnostic unit, or medical imaging department.

If consent has not been obtained for a non-emergency treatment/procedure and the patient has been ordered a premedication the RN is to contact the Surgeon/ Interventionist/ Anaesthetist, if applicable/ Clinical Nurse Manager/ Clinical Nurse Specialist/ Consultant to advise them that the patient cannot be given the pre-operative sedation because there is no consent form for the proposed procedure.

The RN is to document the reason for withholding the pre-operative medication in the patient's health record. Essential non-sedative medication may be given to the patient at this time on instruction from a responsible Medical/ Nurse Practitioner.

Consent is valid until the patient withdraws consent or there is a change in the patient's circumstances such as an improvement or deterioration, development of new treatment options or progression of the disease, which may have changed the treatment from cure to palliation.

Where telehealth has been used for consent discussions, the consent process must still occur in full.

Consent must be reviewed by a medical practitioner or HP if consent was obtained more than three months prior to the treatment being performed. Evidence of the review must be evident in the health record.

2. Documentation

Written consent, using an approved consent form, must be obtained for, surgical, medical, radiology, oncology and endoscopic treatments/ procedures requiring general anaesthesia, regional anaesthesia, or intravenous sedation.

Documentation of consent is essential and a written record of the key points from discussions of the proposed treatment and consent decisions must be documented in the consent form regardless of whether the patient consents, declines, or withdraws consent to the proposed treatment.

The minimum documentation requirements.

- Any communications needs identified
- Procedures/ proposed treatment (including whether anaesthesia is required)
- The correct site and anatomical location on which the procedure is to be performed.
- Printed name/signature of the health practitioner
- Printed name/signature of the patient
 - **For Telehealth appointments** – The health practitioner that undertook the consent discussion must annotate the consent form on behalf of the patient, noting that the discussion occurred via Telehealth. This consent must be confirmed with the patient's own signature (or that of their substitute decision maker) prior to treatment commencing.
- Acknowledgement of alternative treatment options.
- Acknowledgement of General Risk disclosure which includes all risks that might affect a patient's treatment decisions.
- Material risk specific to the patient.
- Record of any benefits, outcomes discussed with the patient.
- List of any information resources provided to the patient.
- Date of Consent

The relevant health practitioner is to ensure that consent has been obtained at each of the following stages:

- At the pre-admission/outpatient clinic, where possible.
- At the patient's admission to hospital, where possible.
- Prior to the administration of preoperative sedative medication.
- Prior to transfer to the operating theatre suite/procedural unit, diagnostic unit, or medical imaging department.

3. Treatment in an Emergency

Treatment in an emergency occurs where urgent treatment is required to save a patient's life or prevent serious harm. If the patient is **competent** there is a requirement to address the consent process within the time constraints of necessity and the patient's best interest.

Although in this circumstance consent is implied, and a formal consent form is not required, details of the medical emergency and communication with the patient must be clearly documented in the patient's health record.

If the patient is **not competent** or unable to provide consent to treatment required to save life or prevent serious deterioration in their condition it may be given without consent provided that:

- the patient is not able to consent.
- there is no overriding Advance Health Directive (AHD) or common law directive that is known and immediately available
- no substitute decision maker is immediately available.

The circumstances of the emergency and the patient's lack of competency to consent are to be clearly documented in the patient's health record, by the most senior doctor present.

In the Intensive Care Unit (ICU), consent for urgent medical treatment should be obtained from the treatment decision maker however, if that is not possible treatment may be given in accordance with the [Guardianship and Administration Act 1990 Section 110ZD](#).

If the patient becomes competent, he / she should be informed about the treatment and the reasons for providing it.

4. Provision of Emergency Psychiatric Treatment

Mental Health patients may receive emergency psychiatric treatment in accordance with [Mental Health Act 2014, Part 14, Division 2 sections 202-204](#).

Refer to the DoH [Operational Directive MP0175/22, WA Health Consent to Treatment Policy](#) or contact the Office of the Chief Psychiatrist for specific information relating to consent for emergency psychiatric treatment.

5. Provision of Medical Treatment Under the Guardianship and Administration Act 1990

The Guardianship and Administration Act 1990 applies to the provision of medical and dental treatment to persons 18 years or over who are either represented by a guardian or are persons for whom a guardian could be appointed. Refer to the DoH [Operational Directive MP0175/22, WA Health Consent to Treatment Policy](#)

6. Treatment for Children and Mature Minor

Children are assumed not to have capacity for consent. Where possible, children should be supported to participate in discussions and decision-making about their treatment, even where they do not have the capacity to make final decisions. The parents of a child who is a minor are usually the appropriate persons to give consent to medical treatment.

A minor is a person under 18 years of age. Refer to, DOH [Operational Directive MP0175/22, WA Health Consent to Treatment Policy](#)

7. Anaesthesia

If general, regional, local anaesthesia or intravenous sedation is to be used for a planned treatment /procedure, the patient must be informed about any material risks associated with the anaesthesia and the advantages and disadvantages of alternative treatments. Where alternative anaesthetic treatment options exist, the advantages and disadvantages of each alternative treatment option should also be discussed.

Information about anaesthesia should be provided to the patient where possible in advance in the pre-admission clinic, outpatient clinic or on admission to the hospital. The patient must be given an opportunity to discuss anaesthesia with an anaesthetist prior to transfer to the operating theatre/procedure room.

Any information given to patient and the details of relevant discussions with the patient must be documented in patient's health record.

8. Open Access Procedures

Consent is required for open access procedures such as endoscopy. MP who refers patients to endoscopy services have a duty of care at the time of referral to provide information about rare but possible complications of the procedure, including risks associated with anaesthesia. The patient's consent for the proposed procedure is to be documented in their health record.

For day procedure patients, information should be provided by the General Practitioner at

the time of referral. In addition, the booking clerks will forward written information about the procedure (including risks) to the patient by post with their appointment date and preparation instructions. Questions may be clarified by the MP performing the procedure at the time of admission to the endoscopy unit. Consent should be appropriately documented using the 'Consent to Treatment' at this time by the MP performing the procedure.

For inpatients, informed consent should be obtained by the MP who has sufficient knowledge of the procedure and its risks prior to transfer to the endoscopy unit and documented on the 'Consent to Treatment Form.'

9. Obstetric Procedures

Consent for obstetric procedures should be obtained in accordance with the requirements set out in Elective patients' section of this Policy. If an emergency caesarean section is required, refer to the [OPH Women's and Newborn Services Caesarean Section- Non-Elective Clinical Practice Guideline](#)

If an urgent blood transfusion is required, the consent to treatment process outlined in Blood production section of this policy should be followed. If spinal/epidural anaesthesia/analgesia is required, refer to Anaesthesia Section of this policy. Where practicable, women should be given written information about epidural anaesthesia/analgesia to read prior to transfer to the Birth Suite.

10. Blood Products

A patient must be advised about the risks associated with receiving a blood transfusion to provide consent. A record of this discussion and information provided to the patient should be documented in the patient medical record. Refer to [SCGOPHCG Informed Transfusion Consent Manual](#).

11. Medications

Non-TGA approved medications, those not being registered with the TGA and used for an indication not registered with the TGA and that no approved indication on the [State-Wide Medicines Formulary](#), must document consent by prescribing MP in the patients integrated progress notes.

12. Consent for a Course of Treatment

Where a course of treatment is required, e.g., Chemotherapy, a single consent to treatment form may be completed. The consent to treatment form must specify that it is for the entire course of treatment. The health practitioner is to document following information on the consent form and in the patient's health record:

- The elements of the course of treatment and any associated material risk.
- Any alternative treatments available.
- The consequences of withdrawal at a future date.

13. Consent for Culturally/Linguistically Diverse Background

Health practitioners must use Language Services interpreting and translations for people who are not able to communicate effectively in written and/or spoken English. This may include people who are deaf, hard of hearing, Aboriginal Australians, and people from culturally and linguistically diverse backgrounds. This is in accordance with the DOH Operational Directive MP0051/17 [WA Health System Language Services Policy](#).

- Staff are to use an appropriately accredited health trained interpreter to assist

patients with special needs or language requirements to understand the procedure and the relevant implications.

- The interpreter is to inform the health practitioner of any concerns he/ she has in relation to the accuracy of the interpretation. If the interpreter considers that he/she is unable to accurately translate the information an alternative interpreter should be obtained. However, if the health practitioner requests the original interpreter to continue this must be documented in the patient's health record.
- if the patient has vision or hearing impairment or is illiterate, the HP obtaining consent must use appropriate communication methods or enlist the support of a person who is able to assist in communicating with the patient.(for more details refer to [WA Health System Language Services Policy](#). The HP is responsible for documenting this in the patient's health record.

14. The Law Either permits or Forbids Treatment

Treatments, which may only be undertaken provided certain requirements are met, must comply with the prescribed requirements. Refer to page 7 of the [WA Health Consent to treatment Procedure MP0175/22](#)

15. Consent for Participation in Health or Human Research

Consent is required for participation in health or human research. Consent is to be obtained in accordance with requirements outlined in the.

- [National Health and Medical Research Council \(NHMRC\)](#)
- [SCGOPHCG Research Governance Policy](#)
- [WA Health MP 0162/21 Research Governance Policy](#)

The recruitment of a participant unable to provide informed consent on their own behalf should be managed in accordance with the requirements of the [Guardianship and Administration Act \(Amendment\)](#) as well as local policy, process or procedure.

16. Patients arriving in the Operating Suites, Procedural Unit, Diagnostic Unit, Or Medical Imaging Department / without valid or documented consent

If the clinical team determines the patient requires emergency treatment to be provided without delay, to save the life/limb of the patient, to reduce pain, or for the patient's ultimate well-being, then the patient is deemed by law to have consented to the treatment.

In this instance, if two specialist practitioners agree that the emergency treatment is life/limb saving and there is no AHD on file, then the surgery can proceed in the patient's best interest. The decision to proceed with the treatment must be clearly documented in the patient's medical record including the necessity to treat, the consequences of not treating, and all attempts made to contact NOK. The DCS and/or medical Executive on call can be contacted for advice if needed.

If the patient is for elective surgery the Operating Suite staff are to contact the first available health practitioner in order of priority to advise that any preoperative sedation ordered cannot be given and the procedure cannot proceed without consent.

All incidents of patients arriving in the operating theatre or treatment room without valid or documented consent must be reported to the relevant Clinical Nurse Manager and entered in to the Datix Clinical Incident Management System.

17. Refusal to Consent to Treatment

If a mentally competent patient refuses to sign a written consent form, then the patient's treatment should not proceed until consent has been validly obtained.

18. Withdrawal of Consent to Treatment

If a patient consents to a procedure but subsequently withdraws consent the date of the withdrawal must be documented in the patient's health record.

It is critical that the patient is made aware that reconsideration of treatment or withdrawal of treatment is always an option, and any changes are to be documented in the patient's health record.

19. Advance Health Directives

Any person who has reached the age of 18 years and who has full legal capacity may make an Advance Health Directive (AHD). Refer to.

- [The WA Health Consent to Treatment Policy](#)
- [WA Hierarchy of Treatment Decision-Makers](#)
- [A National Framework for Advance Health Directive \(AHD\)](#)

20. Consent for Photographs / Video of Patients

Written consent must be obtained from the patient when the image is to be used for any publication or display purposes including submissions to a medical journal, patient information publications, posters, PowerPoint presentations, media - newspaper, television news etc. All signed consent forms are to be retained in the patient's health record.

Refer to [SCGOPHCG Clinical Photography and Audio-Visual Recordings](#)

21. Financial Consent

When a patient is admitted to hospital their patient classification will determine whether any fees or charges will be raised for hospital accommodation or other services.

The health practitioner/ Admission staff/ Private Patient Liaison Officer obtaining the financial consent from the patient is to ensure that all relevant documentation is completed.

Where a patient's admission will incur fees or other charges financial consent is required for payment for treatment/service and this is to be obtained in accordance with relevant legislation and Government/ Department of Health policies.

Compliance, monitoring and evaluation

SCGOPHCG Executive Director to ensure compliance with this policy.

NSQHS Partnering with Consumers Committee conducts routine audits on consent as part of their audit schedule.

Audit of consent will ensure compliance with legislation and best practice.

Reporting of compliance will be through to the SCGOPHCG Clinical Governance Committee.

Related Documents

- [Guardianship and Administration Act 1990 Section 110ZD.](#)
- [WA State Records ACT](#)

- [Mental Health Act 2014, Part 14, Division 2 sections 202-204.](#)
- [MP 0175/22 WA Health Consent to Treatment Policy.](#)
- [MP 0175/22 WA Health Consent to treatment Procedure](#)
- [MP0051/17 WA Health System Language Services Policy.](#)
- [National Health and Medical Research Council \(NHMRC\)](#)
- [SCGOPHCG Research Governance Policy](#)
- [WA Health MP 0162/21 Research Governance Policy](#)
- [SCGOPHCG Informed Transfusion Consent Manual.](#)
- [OPH Women's and Newborn Services Caesarean Section- Non-Elective Clinical Practice Guideline](#)
- [Flowchart - Can your patient consent to treatment?](#)
- [Fact Sheet for clinicians- Informed consent in health care](#)

Related forms

Consent for Procedure – Colonoscopy, MR703.8
 Consent for Procedure – Gastroscopy, Colonoscopy MR703.8
 Consent for Procedure – Gastroscopy, MR703.8
 Consent for Radiotherapy Treatment, 400.1
 Consent to Blood Products, Form C, 007/MR(OPH)30A
 Refusal to Consent to Blood Products (Form D) 007/MR(OPH)30B
 Consent to Dialysis Treatment, MR730.8
 Consent to Treatment (Form A) MR730.81/ MR(OPH)30
 Consent to General Anaesthesia +/- Regional Anaesthesia (Form B) 730.82
 Sotrovimab (COVID-19) Prescription/Consent, MR809.4
 Non-Vented Checklist and Consent, MR383
 Consent to General Anaesthesia +/- Regional Anaesthesia, MR (OPH)30F
 Patient Photography Request & Consent 398.1
 Restraint / Consent Observation Chart MR145

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